

Atopic Eczema Primary Care Protocol (Includes Children)

Version 6.2: September 2018
Next review due: Post Covid

Key

- "Must do" actions for GP's / (Triaged by RMS)
- Recommendations for Primary Care
- Red flag / urgent referral
- Routine referral
- Public health intervention
- Audio-visual aids for patients and GP
- Click icon for clinical evidence

Patient presents with childhood atopic eczema

History

- Presence of itching?
- Starting in infancy & episodic?
- Family history of atopy?

Examination

- Localised to flexure of limbs in children
- Involving face, scalp, extensor surface of limbs in infants
- Chronic eczema is characterised by thickened skin resulting from repeated scratching

Triggers

- Irritant Allergies – soaps, detergents
- Irritant Clothing – synthetics and wool and sometimes silk
- Contact Allergies – preservatives in topical medicines, perfume-base products, metals, latex.
- Inhaled Allergies – pets, pollen
- Climate – summer (very hot), winter (very cold)
- Dietary – Itch/redness immediately or hours after food and can include diarrhoea and vomiting e.g. milk, eggs, wheat, soy, peanut (75% cause of food induced atopic eczema)

RED FLAG SYMPTOMS

- Eczema herpeticum – fever, lymphadenopathy, malaise
 - Herpes simplex – grouped vesicles and punched out erosions
 - Infection that has not responded to treatment after 2-3 weeks (e.g. treatment with Flucloxacillin or Erythromycin – remember to do a swab)
- [Recurrent infections/abscess/ history of associated pneumonia – arrange routine referral]*

For children <6 months of age where diet is thought to be the cause?

Consider 6-12 week trial of hydrolysed protein formula milk (e.g. Aptamil Pepti or Nutramigen) or amino acid milk (e.g. Necoate LCP)

Mild

1. Emollients (e.g. Epimax TDS/QDS, Zerobase)
2. Mild steroid (e.g. Hydrocortisone 1% OD/BD)

Moderate

1. Emollients (e.g. Isomol TDS/QDS)
2. Moderate steroid (e.g. Betamethasone valerate 0.025% **OR** Clobetasone butyrate 0.05% & for delicate areas e.g. face/flexures use mild steroid e.g. Hydrocortisone 1%)
3. If itching consider prescribing antihistamines (e.g. cetirizine)

Severe

1. Emollients (e.g. Isomol TDS/ODS)
2. Potent steroid (e.g. Betamethasone valerate 0.1% and for delicate areas Betamethasone valerate 0.025% or clobetasone butyrate 0.05% for 5 days)

Refer all patients with severe eczema

Refer if symptoms not controlled, >2 flares per month, facial eczema not responding or recurrent infection

Refer to Community Dermatology via RMS

Refer to Paediatric Allergy Clinic via RMS

** Think about allergies and triggers in over 2 year olds**